

TROUTLY ANALYTICS, INC.

Group Medical Plan

Plan Document and Summary Plan Description

Plan Sponsor:	Troutly Analytics, Inc.
Plan Name:	Troutly Analytics, Inc. Group Medical Plan
Plan Number:	501
Plan Year:	September 1, 2025 through August 31, 2026
Effective Date:	September 1, 2025
Prepared By:	Lampasas Benefits Group

This document constitutes both the Plan Document and the Summary Plan Description required under the Employee Retirement Income Security Act of 1974, as amended.

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Section 1. Introduction and Plan Sponsor Information

1.1 Establishment and Purpose

Troutly Analytics, Inc. (the "Plan Sponsor" or the "Company") maintains the Troutly Analytics, Inc. Group Medical Plan (the "Plan") for the exclusive benefit of eligible employees and their eligible dependents. This document constitutes both the Plan Document and the Summary Plan Description ("SPD") required under the Employee Retirement Income Security Act of 1974, as amended ("ERISA"), and describes the terms, conditions, benefits, limitations, and administrative procedures that govern the Plan for the plan year beginning 09/01/2025 and ending 08/31/2026.

The Plan is a welfare benefit plan that provides medical, hospital, surgical, prescription drug, and preventive care coverage to Covered Persons through a fully insured contract of insurance issued to the Plan Sponsor. Benefits are underwritten and paid by the contracted insurance carrier under a group insurance policy, and this document should be read together with the Certificate of Coverage, the Schedule of Benefits, and any endorsements or riders issued by the carrier for the plan year.

Nothing in this document creates a contract of employment or a guarantee of continued employment. The Plan Sponsor reserves the right to amend, modify, suspend, or terminate the Plan in whole or in part at any time by action of its Board of Directors or a duly authorized officer, subject to the terms of the applicable insurance contract and to any accrued rights of Covered Persons as of the effective date of the change.

1.2 Plan Sponsor

Legal Name:	Troutly Analytics, Inc.
Principal Address:	2405 South Congress Avenue, Austin, TX 78704
Telephone:	(512) 555-0100
State of Incorporation:	Delaware
Business:	Marketing analytics software (SaaS)
Employer Identification Number:	on file with the Plan Administrator

The Plan Sponsor is the named fiduciary of the Plan for purposes of ERISA section 402(a), except with respect to those benefit determinations and claims decisions that are the responsibility of the insurance carrier under the Certificate of Coverage.

1.3 Plan Name and Identification

Plan Name:	Troutly Analytics, Inc. Group Medical Plan
Plan Number:	501
Type of Plan:	Group health plan providing medical, hospital, surgical, and prescription drug benefits, fully insured
Type of Administration:	Insurer administration for benefit determinations and claim payments; Plan Sponsor administration for eligibility, enrollment, contributions, and general Plan administration

1.4 Plan Year

The Plan Year begins 09/01 and ends 08/31 of each calendar year. The Plan Year covered by this document runs from 09/01/2025 to 08/31/2026. All accumulators used to determine benefit thresholds — including deductibles, out-of-pocket maximums, benefit-year maximums, and visit limits — reset to zero on 09/01 of each Plan Year and accumulate through 08/31 of the following calendar year. The ERISA plan year for reporting and disclosure purposes coincides with the Plan Year.

1.5 Plan Administrator

The Plan Administrator is the Head of People of Troutly Analytics, Inc. The Plan Administrator may be reached at the Plan Sponsor's principal address and telephone number above. The Plan Administrator has the discretionary authority to:

- interpret the terms of the Plan;
- determine eligibility for participation and benefits, other than benefit determinations reserved to the insurance carrier;
- adopt rules and procedures necessary or appropriate for the administration of the Plan;
- prescribe the form and content of all notices, elections, and disclosures required by the Plan; and
- retain and supervise service providers, including the insurance carrier, the broker of record, third-party administrators, and legal counsel.

Decisions of the Plan Administrator within the scope of its discretionary authority are final and binding on all persons interested in the Plan, subject to the claims and appeals procedures set forth in Section 7 and, where applicable, external review under the Affordable Care Act.

1.6 Agent for Service of Legal Process

Legal process may be served on the Plan Administrator at the Plan Sponsor's principal address set forth in Section 1.2. Service of legal process may also be made upon the Plan Sponsor's registered agent in the state of incorporation.

1.7 Broker of Record

The Plan Sponsor has appointed Lampasas Benefits Group as its broker of record and consultant for the Plan Year. Lampasas Benefits Group assists the Plan Sponsor with carrier selection, renewal analysis, rate negotiation, contract administration, open enrollment communications, and compliance support. The primary contact at Lampasas Benefits Group for the Plan Sponsor is the assigned Benefits Broker, whose contact information appears on the annual rate sheet, the enrollment guide, and the summary of material modifications distributed for the Plan Year. Lampasas Benefits Group is not a fiduciary of the Plan and does not adjudicate claims.

1.8 Insurance Carrier

Benefits under the Plan are provided under a group insurance contract issued by the contracted medical carrier for the Plan Year. The Certificate of Coverage issued by the carrier is incorporated by reference into this document and controls the availability and amount of benefits, network access, medical necessity determinations, prior authorization requirements, and clinical policies. In the event of a conflict between this Plan Document and the Certificate of Coverage on a matter of benefit determination, the Certificate of Coverage controls; on a matter of eligibility, enrollment, or contribution, this Plan Document controls.

1.9 Funding

Benefits under the Plan are funded exclusively through the group insurance contract. Premiums are paid to the insurance carrier from the general assets of the Plan Sponsor and from employee payroll contributions, which are held as general assets of the Plan Sponsor pending remittance to the carrier and are not held in trust. The Plan does not maintain a trust fund and does not fund benefits on a self-insured basis.

1.10 Governing Law

The Plan is governed by ERISA and, to the extent not preempted, by the laws of the State of Texas. The insurance contract funding the Plan is governed by the Texas Insurance Code and applicable federal law. Where a provision of the Plan is or becomes invalid or unenforceable, the remaining provisions continue in full force and effect.

Section 2. Eligibility

2.1 Employee Classes

The Plan is offered to the following classes of employees of the Plan Sponsor who satisfy the general eligibility conditions set forth in this Section 2:

Class A — Regular Full-Time Employees. Employees classified by the Plan Sponsor in its personnel records as regular full-time and regularly scheduled to work 30 or more hours per week on a continuing basis.

Class B — Regular Part-Time Employees Meeting the Hours Threshold. Employees classified as regular part-time whose position has been formally revised to a regularly scheduled workweek of 30 or more hours. A prior part-time employee whose regular schedule is increased to 30 or more hours becomes eligible on the date the increased schedule takes effect and completes coverage in accordance with Section 2.3.

Excluded Classes. The following persons are not eligible to participate in the Plan, regardless of hours worked, tenure, or method of compensation:

- Independent contractors, consultants, and persons engaged through a contract for services, whether or not later reclassified as employees by any court, agency, or taxing authority. Reclassification does not retroactively confer eligibility for any period prior to the date the reclassification is accepted in writing by the Plan Administrator.
- Leased employees within the meaning of Internal Revenue Code section 414(n).
- Temporary employees engaged for a defined project or period of less than six months, whether hired directly or through a staffing agency.
- Interns and co-op students, whether paid or unpaid.
- Employees covered by a collective bargaining agreement, unless the agreement expressly provides for participation in this Plan.
- Non-resident aliens with no United States source income.
- Employees classified as seasonal.

Classification for purposes of this Plan is determined solely by the personnel records of the Plan Sponsor as maintained by the Plan Administrator in the human resources information system.

2.2 Hours Threshold

For all purposes of this Plan, the hours threshold for eligibility is 30 or more hours per week as reflected in the employee's regularly scheduled workweek recorded in the Plan Sponsor's human resources information system. An employee whose scheduled hours are reduced below the threshold ceases to satisfy the eligibility requirement as of the effective date of the reduction, subject to the coverage-end and continuation provisions of Section 6.

The threshold is measured as a regularly scheduled workweek and not by reference to the hours worked in any single week or pay period. Occasional variation in actual hours worked above or below 30 in a given week does not, by itself, alter eligibility so long as the position remains regularly scheduled at or above the threshold. A written change to the employee's regular schedule that reduces the position below 30 hours will end eligibility in accordance with Section 6.

Employees classified by the Plan Sponsor as variable-hour or whose hours cannot be determined at the date of hire are evaluated under a look-back measurement method consistent with the Affordable Care Act employer shared-responsibility regulations, using an initial measurement period of twelve consecutive months beginning on the first day of the calendar month following the date of hire, a stability period of twelve months, and an administrative period of not more than 60 days between the measurement and stability periods. An employee determined during the initial or a standard measurement period to have averaged 30 or more hours per week is treated as satisfying Section 2.1 and Section 2.3 for the corresponding stability period.

2.3 Effective Date of Coverage

An employee regularly scheduled 30 or more hours per week is eligible for coverage effective the first day of the month following 30 days of eligible employment. This waiting period runs from the employee's date of hire as recorded in the human resources information system and satisfies the 90-day maximum waiting period under Public Health Service Act section 2708 and Affordable Care Act section 1201.

For purposes of applying the waiting period, the 30-day count runs on a calendar-day basis, includes weekends and holidays, and is measured from and including the employee's first day of eligible employment. Coverage becomes effective at 12:01 a.m. Central Time on the first day of the calendar month that follows the day on which the 30-day period is completed. Where an employee's date of hire is the first day of a calendar month, the 30-day period nonetheless runs from that date, and coverage becomes effective on the first day of the next following calendar month. Where an employee is hired on any day other than the first of the month, no partial-month coverage is provided.

An employee who timely elects coverage within the enrollment window described in Section 3.2 will be enrolled effective as of the date determined under this Section 2.3. An employee who fails to complete enrollment within the applicable window is treated as having declined coverage for the balance of the Plan Year, subject to the qualifying life event and special enrollment rules described in Section 3.4.

Movement between excluded and eligible classes during employment is treated as a new eligibility event on the effective date of the class change. An employee who transfers into an eligible class from a previously excluded class is eligible for coverage effective the first day of the month following 30 days of eligible employment in the eligible class, applied from the date of transfer.

Nothing in this Section 2.3 shortens or lengthens the coverage effective date for an employee whose eligibility arises from a change described in Section 2.2 (change in scheduled hours) or Section 2.5 (rehire), for whom the specific rules of those sections apply.

2.4 Dependents

An eligible employee may enroll the following eligible dependents:

Spouse. The employee's lawful spouse under the laws of the state in which the marriage was contracted. Common-law spouses recognized under applicable state law are eligible upon submission to the Plan Administrator of a declaration of informal marriage on the form prescribed by the State of Texas or equivalent documentation acceptable to the Plan Administrator.

Children. Each child of the employee, up to age 26, without regard to marital status, financial dependency, residence with the employee, student status, or eligibility for other coverage. For this purpose a "child" is:

- a natural child;
- a legally adopted child, including a child placed with the employee for adoption;
- a stepchild residing with the employee or the employee's spouse;
- a child for whom the employee or the employee's spouse has been appointed legal guardian; and
- a child for whom coverage is required by a Qualified Medical Child Support Order ("QMCSO") issued by a court of competent jurisdiction and determined by the Plan Administrator to satisfy the requirements of ERISA section 609.

Disabled Adult Children. A child who has attained age 26 may continue to be covered under the Plan without regard to the age limit if the child is and continues to be incapable of self-sustaining employment by reason of mental or physical disability, and the child became so disabled prior to attaining age 26. Proof of disability must be submitted to the insurance carrier within 31 days of the child's 26th birthday and periodically thereafter as the carrier may require.

Documentation. The Plan Administrator may require the employee to furnish documentation of dependent status before enrollment is completed, including but not limited to marriage certificates, birth certificates, adoption or placement orders, court orders of guardianship, and, in the case of a QMCSO, a certified copy of the order. Coverage of a person as a dependent who does not meet the definitions in this Section 2.4 is subject to rescission and recovery of benefits paid on the person's behalf.

Ineligible Dependents. The following are not eligible dependents: former spouses (whether or not required to be covered under a divorce decree, except as required by a QMCSO for children); parents, grandparents, siblings, cousins, and other extended family members; foster children not placed for adoption; and any person residing outside the United States other than temporarily.

2.5 Rehires

A former employee who is rehired into an eligible class within 13 weeks of the last day of prior eligible employment shall be treated as continuing prior service for purposes of the waiting period in Section 2.3 and shall become eligible for coverage on the later of (i) the date of rehire or (ii) the date on which the employee would have become eligible under Section 2.3 had employment not been interrupted, provided that the employee had satisfied or was satisfying the waiting period at the time of the prior separation.

A former employee who is rehired more than 13 weeks after the last day of prior eligible employment shall be treated as a new hire for all purposes of the Plan, including satisfaction of the waiting period in Section 2.3, and coverage will not begin earlier than the first day of the month following 30 days of eligible employment measured from the rehire date.

Where a rehired employee had elected COBRA-equivalent continuation coverage under Section 6 following the prior separation and that continuation coverage remains in effect at the time of rehire, the employee's continuation coverage will end as of the effective date of active coverage under this Section 2.5.

Prior service for purposes of vacation accrual, seniority, or other personnel matters is governed by the Company's separate service-bridging policies and is not affected by the eligibility rule of this Section 2.5.

Section 3. Enrollment

3.1 Coverage Tiers

Each eligible employee may elect one of the following coverage tiers for the Plan Year:

- **Employee Only** — coverage for the employee only.
- **Employee plus Spouse** — coverage for the employee and the employee's lawful spouse.
- **Employee plus Children** — coverage for the employee and one or more eligible children of the employee.
- **Family** — coverage for the employee, the employee's lawful spouse, and one or more eligible children of the employee.

An employee may also decline (waive) coverage under the Plan. An employee who waives coverage must complete the waiver acknowledgement in the enrollment system and will not be enrolled for the Plan Year unless a qualifying life event permits mid-year enrollment under Section 3.4. Waiver of coverage does not by itself entitle the employee to any cash payment, opt-out credit, or other consideration in lieu of coverage.

Enrollment is by employee-elected tier. The Plan does not offer à la carte enrollment of individual dependents or member-level premium pricing. Once the tier is elected, all eligible dependents in the tier definition who are enrolled become Covered Persons at the tier premium.

3.2 Initial and New-Hire Enrollment

A newly eligible employee must complete enrollment in the Plan Sponsor's benefits enrollment system within 30 days of the date the employee first satisfies Section 2.1. Enrollment includes:

- election of a coverage tier or a waiver;
- identification of each dependent to be covered under the elected tier; and
- authorization for payroll deduction of the employee share of the applicable premium.

An employee who does not complete enrollment within the 30-day window is treated as having declined coverage for the Plan Year and may enroll only during the next Open Enrollment period or upon the occurrence of a Qualifying Life Event.

3.3 Open Enrollment

The Plan Sponsor conducts an Open Enrollment period each year in advance of the beginning of the Plan Year on 09/01. The Open Enrollment window is a period of not fewer than 14 calendar days ending not later than seven calendar days before 09/01. The specific dates of Open Enrollment are announced by the Plan Administrator in advance through the enrollment system and the annual open enrollment communications distributed with the assistance of Lampasas Benefits Group.

During Open Enrollment, each eligible employee may:

- enroll if not currently enrolled;
- change coverage tier;
- add or drop eligible dependents; or
- waive coverage for the coming Plan Year.

Elections made during Open Enrollment are effective 09/01 of the new Plan Year and remain in effect through 08/31 of the Plan Year, unless changed on account of a Qualifying Life Event during the Plan Year. An employee who does not affirmatively make an election during Open Enrollment shall be deemed to have continued in effect the same coverage tier (or waiver) that was in effect on 08/31 of the prior Plan Year, with premiums adjusted to the rates in effect for the new Plan Year.

3.4 Qualifying Life Events and Special Enrollment

A Covered Person or eligible employee may enroll or change coverage outside of Open Enrollment upon the occurrence of one of the following qualifying life events, provided that the requested change is consistent with the event and is timely requested:

- **Marriage** — the employee may add a spouse and, if desired, change tiers to include the spouse.
- **Birth** — the employee may add a newborn child and, if not already enrolled in a tier that includes children, change tiers to include the child.
- **Adoption** — the employee may add an adopted child or a child placed for adoption and change tiers accordingly.
- **Divorce** — the employee must drop the former spouse and any stepchildren who cease to be eligible dependents; the employee may change tiers consistent with the loss of the dependents.
- **Dependent Aging Out** — an employee must drop a child who has reached the limiting age of 26 (subject to the disabled adult child provisions of Section 2.4) and may change tiers accordingly.
- **Loss of Other Coverage** — an employee or dependent who involuntarily loses other group health coverage may enroll under the Plan.

Enrollment changes on account of a qualifying life event require the employee to submit the change request in the enrollment system, together with supporting documentation, **within 30 days** of the date of the event. Requests submitted after the 30-day window will not be processed, and the affected person will remain ineligible to enroll until the next Open Enrollment. Newly added dependents are covered effective the date of the event (retroactive to the event for births, adoptions, and placements for adoption); dependents dropped because of divorce or aging out are removed as of the date of the event, subject to the COBRA-equivalent continuation rights described in Section 6.

Special enrollment rights under HIPAA and the Children's Health Insurance Program Reauthorization Act ("CHIPRA") are preserved in accordance with applicable law and are applied consistent with the 30-day (or, in the case of CHIPRA events, 60-day) notice window prescribed by those statutes.

3.5 Effect of Failure to Enroll

Enrollment in the Plan is affirmative and elective. No employee is automatically enrolled. Failure of an eligible employee to complete initial enrollment within 30 days of first becoming eligible under Section 2.3, or to make a timely election during Open Enrollment, or to submit a timely qualifying life event request under Section 3.4, results in the employee having no coverage under the Plan until the next Open Enrollment or the next qualifying life event, whichever occurs first.

Section 4. Contributions

The company pays 80 percent of the Employee Only monthly premium toward every enrolled employee's coverage regardless of tier. The employee pays the balance of their tier's premium through payroll deduction. Premium amounts are set by the annual rate sheet.

The employer contribution described in the preceding paragraph is a fixed monthly dollar amount equal to 80 percent of the Employee Only tier premium published on the rate sheet for the Plan Year, and is the same dollar contribution for every enrolled employee irrespective of whether the employee has elected Employee Only, Employee plus Spouse, Employee plus Children, or Family coverage. The employee's share of premium for any Plan Year is the full monthly premium for the tier elected less the employer contribution.

Employee contributions are collected through payroll deduction over the pay periods occurring in each month of coverage. Deductions are taken on a pre-tax basis under a cafeteria plan maintained by the Plan Sponsor under Internal Revenue Code section 125, unless the employee has affirmatively elected in writing to have contributions taken on an after-tax basis. Elections to change the pre-tax or after-tax character of contributions may be made only during Open Enrollment or upon a qualifying life event that affects the tax election consistent with Treasury regulations under section 125.

Where the Plan Sponsor's payroll cycle is biweekly (26 pay periods per year), monthly premium amounts are converted to per-pay-period deductions by multiplying the monthly premium by 12 and dividing by 26. Where an employee begins or ends coverage mid-month by reason of an event other than 09/01 or 08/31 (for example, on the effective date established under Section 2.3), the employee's contribution for the partial period is prorated in accordance with the payroll conventions in effect for that Plan Year.

Waived employees make no contribution. Continuation-coverage participants under Section 6 pay the full monthly premium plus any applicable administrative surcharge, without any employer contribution.

The annual rate sheet is issued by Lampasas Benefits Group on behalf of the Plan Sponsor in connection with each renewal and forms part of the Plan's enrollment materials. The rate sheet in effect at the beginning of the Plan Year controls contributions for the Plan Year unless the Plan Sponsor issues a mid-year amendment.

Section 5. Schedule of Benefits

The following Schedule of Benefits describes cost-sharing and benefit limits applicable under the Plan for the Plan Year. All amounts are stated in United States dollars. Benefit-year accumulators reset on 09/01 of the Plan Year and accumulate through 08/31 of the following calendar year. All benefits are subject to medical necessity, utilization review, and prior authorization requirements described in the Certificate of Coverage.

5.1 Deductibles

A Deductible is the amount a Covered Person must pay for Covered Services in a Plan Year before the Plan begins to pay coinsurance for those services. The Deductible does not apply to services identified in this Section 5 as covered at 100% without regard to the Deductible (including in-network preventive care as described in Section 5.5).

- **Individual Deductible (In-Network):** \$2,000 per Covered Person per Plan Year.
- **Family Deductible (In-Network):** \$4,000 per family unit per Plan Year, satisfied on an aggregate basis by any combination of covered family members. Once the family Deductible is satisfied, the Deductible is deemed satisfied for every covered family member for the remainder of the Plan Year.
- **Out-of-Network Deductibles:** are set forth in the Certificate of Coverage and are generally higher than the in-network amounts. Amounts accumulated toward the in-network Deductible do not accumulate toward the out-of-network Deductible, and vice versa.

Amounts paid toward the Deductible must be for Covered Services and must be paid to a provider (or to the carrier by the Covered Person) as evidenced by an explanation of benefits. Balance billing for out-of-network services (other than protected services under the No Surprises Act) does not accumulate toward the Deductible.

5.2 Coinsurance

After the Deductible has been satisfied, the Plan pays a percentage of the negotiated (in-network) or allowed (out-of-network) charge for Covered Services, and the Covered Person is responsible for the remaining percentage as coinsurance:

- **In-Network Coinsurance:** the Plan pays 80% and the Covered Person pays 20% of allowed charges.
- **Out-of-Network Coinsurance:** the Plan pays a lower percentage and the Covered Person pays a higher percentage, as set forth in the Certificate of Coverage. Out-of-network coinsurance does not accumulate toward the in-network out-of-pocket maximum except with respect to services protected by the No Surprises Act.

5.3 Out-of-Pocket Maximums

The Out-of-Pocket Maximum ("OOPM") is the most a Covered Person will pay in Deductibles, copays, and coinsurance for in-network Covered Services in a Plan Year. Once a Covered Person or family satisfies the applicable OOPM, the Plan pays 100% of allowed charges for in-network Covered Services for the remainder of the Plan Year.

- **Individual OOPM (In-Network):** \$6,500 per Covered Person per Plan Year.
- **Family OOPM (In-Network):** \$13,000 per family unit per Plan Year, satisfied on an aggregate basis by any combination of covered family members, subject to the requirement that no single Covered Person's out-of-pocket expenses count toward that person's individual OOPM in excess of the individual OOPM amount.
- **Out-of-Network OOPM:** as set forth in the Certificate of Coverage.

Amounts that do not accumulate toward the OOPM include premium contributions, non-Covered Services, amounts in excess of usual and customary charges for out-of-network services, penalties for failure to obtain required prior authorization, and balance-billed charges other than protected charges under the No Surprises Act.

5.4 Office Visit and Facility Copays

Certain services are subject to a fixed-dollar copay in lieu of, or in addition to, coinsurance, as follows:

Service	Cost-Sharing
Primary Care Physician Office Visit (In-Network)	\$30 copay per visit, not subject to Deductible
Specialist Office Visit (In-Network)	\$60 copay per visit, not subject to Deductible
Urgent Care (In-Network)	\$75 copay per visit, not subject to Deductible
Emergency Room	\$350 copay per visit, plus in-network coinsurance after Deductible; copay waived if admitted inpatient directly from ER
Ambulance (ground and air, emergency)	In-network coinsurance after Deductible; balance billing protected under the No Surprises Act where applicable
Telehealth (In-Network)	\$15 copay per visit for a covered telehealth consultation with a network provider through the carrier's designated telehealth platform
Inpatient Hospital Admission (In-Network)	\$500 per-admission copay, plus in-network coinsurance after Deductible
Outpatient Surgery (In-Network Facility)	In-network coinsurance after Deductible; no separate facility copay

Service	Cost-Sharing
Diagnostic Imaging (MRI, CT, PET, In-Network)	In-network coinsurance after Deductible; prior authorization required
Physical, Occupational, and Speech Therapy (In-Network)	\$40 copay per visit, up to 60 visits per Plan Year combined across the three therapies

Copay amounts count toward the in-network OOPM and, except where specifically stated as "not subject to Deductible," toward the in-network Deductible.

5.5 Preventive Care

The Plan covers in-network preventive care services at 100% of the negotiated charge, without application of the Deductible, copay, or coinsurance, when rendered by an in-network provider and billed as preventive. Preventive care services covered on this basis include, without limitation:

- routine adult and pediatric physical examinations and well-woman visits recommended by the United States Preventive Services Task Force ("USPSTF") at Grade A or B, the Advisory Committee on Immunization Practices ("ACIP"), and the Health Resources and Services Administration ("HRSA");
- routine immunizations for adults and children recommended by ACIP;
- routine screenings for cancer (including breast, cervical, colorectal, lung, and prostate screenings, subject to age and risk-factor criteria in the applicable guidelines), diabetes, hypertension, cholesterol, obesity, depression, and behavioral risk factors;
- prenatal care, breastfeeding support and equipment (including one standard-grade breast pump per pregnancy), and postpartum care;
- contraceptive services and counseling for women as required by HRSA;
- routine pediatric care in accordance with the Bright Futures periodicity schedule; and
- tobacco cessation counseling and pharmacotherapy for adult users.

Services that are diagnostic in nature — including follow-up testing prompted by a preventive screening — are not preventive services under this Section 5.5 and are subject to the Deductible, copay, or coinsurance otherwise applicable. Services rendered by an out-of-network provider are not covered as preventive at 100% and are subject to the ordinary out-of-network cost-sharing rules.

5.6 Prescription Drug Benefit

The Plan provides prescription drug coverage through the pharmacy benefit administered under the group insurance contract. Cost-sharing applies at the point of sale based on a tiered formulary maintained by the pharmacy benefit manager and updated periodically for the Plan Year.

Retail Pharmacy (up to a 30-day supply, In-Network Pharmacy):

Tier	Description	Cost-Sharing
Tier 1	Generic	\$10 copay
Tier 2	Preferred Brand	\$40 copay
Tier 3	Non-Preferred Brand	\$70 copay
Tier 4	Specialty	30% coinsurance, up to \$250 max per fill

Mail-Order Pharmacy (up to a 90-day supply, In-Network):

Tier	Description	Cost-Sharing
Tier 1	Generic	\$25 copay
Tier 2	Preferred Brand	\$100 copay
Tier 3	Non-Preferred Brand	\$175 copay
Tier 4	Specialty	Not available through mail order; must be filled at a designated specialty pharmacy

Preventive Prescription Drugs. Prescription drugs required to be covered without cost-sharing under the Affordable Care Act, including certain contraceptives and tobacco cessation products, are dispensed at \$0 cost-share when a generic or in-network preferred product is selected, in accordance with the Plan's clinical policies.

Prior Authorization and Step Therapy. The pharmacy benefit manager may require prior authorization, step therapy, or quantity limits for certain drugs, including specialty medications and high-cost brand drugs. Denials of coverage on clinical grounds are subject to the claims and appeals procedures in Section 7.

Out-of-Network Pharmacy. Prescriptions filled at out-of-network pharmacies are subject to higher cost-sharing and may require the Covered Person to pay the full charge and file a paper claim for partial reimbursement, subject to the allowed amount under the pharmacy contract.

Prescription drug copays and coinsurance count toward the in-network OOPM but not toward the medical Deductible except with respect to preventive drugs required by the Affordable Care Act.

5.7 Maternity, Mental Health, and Substance Use Disorder Benefits

Maternity care, including prenatal visits, delivery, and postpartum care, is covered on the same basis as other medical care under this Section 5, subject to the Deductible and coinsurance. In-network prenatal office visits recommended under USPSTF or HRSA guidelines are covered at 100% as preventive care under Section 5.5.

Mental health and substance use disorder benefits are covered on a basis that satisfies the requirements of the Mental Health Parity and Addiction Equity Act. Cost-sharing, treatment limitations, and utilization review criteria applied to mental health and substance use disorder benefits are no more restrictive than those applied to substantially all medical/surgical benefits within the same classification.

5.8 Exclusions

The following services and items are excluded from coverage under the Plan, in addition to any exclusions set forth in the Certificate of Coverage: cosmetic surgery except as required to correct a congenital anomaly or to reconstruct following mastectomy under the Women's Health and Cancer Rights Act; custodial care; experimental or investigational treatments except as required by law for clinical trials; services that are not medically necessary; services rendered by a provider related to the Covered Person (spouse, parent, child, sibling) where no charge would otherwise be made; services covered by workers' compensation; services covered by any government program (except as required by law); services rendered before the effective date or after the termination date of coverage; and any other exclusion listed in the Certificate of Coverage.

Section 6. Termination of Coverage

Coverage under the Plan ends on the earliest of the following:

- **Separation from Employment.** Coverage for an employee and any enrolled dependents ends at 11:59 p.m. Central Time on the last day of the calendar month in which the employee's separation from

employment (voluntary or involuntary) is effective, regardless of the day of the month on which separation occurs.

- **Loss of Eligibility Other Than Separation.** For an employee whose regularly scheduled hours are reduced below the threshold in Section 2.2 or who transfers into an excluded class under Section 2.1, coverage ends at 11:59 p.m. Central Time on the last day of the calendar month in which the loss of eligibility takes effect.
- **Loss of Dependent Status.** Coverage for a dependent ends on the last day of the calendar month in which the person ceases to be an eligible dependent (for example, on divorce, on a child reaching the limiting age of 26, or on the death of the employee). The employee must notify the Plan Administrator within 30 days of the event; failure to do so does not extend coverage for the dependent, and any premium paid for a period after loss of eligibility will be refunded net of claims paid on the dependent's behalf.
- **Termination of the Plan.** Coverage ends on the date the Plan is terminated by the Plan Sponsor.
- **Nonpayment of Contributions.** Coverage may be terminated for nonpayment of the employee share of premium, with the effective date determined by the Plan Administrator in accordance with the payroll grace period generally applicable to employee benefit deductions.
- **Fraud or Misrepresentation.** Coverage may be rescinded for fraud or intentional misrepresentation of a material fact by the employee or a Covered Person, subject to the notice requirements of the Affordable Care Act. Rescission may be effective back to the beginning of the coverage or the date the fraud or misrepresentation occurred, and the Plan may recover benefits paid.

6.1 Continuation of Coverage

Troutly Analytics, Inc. employed 20 or more employees on a typical business day in the preceding calendar year, so this Plan is subject to the continuation coverage requirements of Title X of the Consolidated Omnibus Budget Reconciliation Act of 1985 (COBRA). Continuation rights under this Section 6.1 are provided under federal COBRA and are administered by the insurance carrier in accordance with the Certificate of Coverage. On the occurrence of a qualifying event (including termination of employment other than for gross misconduct, reduction in hours below the eligibility threshold, divorce, death of the employee, or a child's loss of dependent status), the affected qualified beneficiary is entitled to elect to continue coverage under the Plan at his or her own expense for the maximum period COBRA allows (generally 18 months, or 36 months for certain events) on the terms described in the election notice. The Plan Administrator will notify the carrier of any qualifying event of which it becomes aware within 30 days of the event, and the carrier will issue the election notice within 14 days of receiving that notice.

Continuation coverage is at the qualified beneficiary's full expense, without any employer contribution, and may include a reasonable administrative surcharge as permitted by law. Failure to elect continuation coverage within the election period specified in the notice, or to timely pay premium, results in termination of continuation rights.

6.2 Notice on Termination of Coverage

Upon termination of coverage, the carrier will provide the Covered Person written notice of the date coverage ended, together with information on the continuation rights described in Section 6.1 and on obtaining other coverage, including through the Health Insurance Marketplace. No certificate of prior coverage is issued, because group health plans may not impose pre-existing condition exclusions under applicable federal law.

Section 7. Claims and Appeals

7.1 Filing a Claim

In-Network Providers. In-network providers file claims directly with the insurance carrier on behalf of the Covered Person. The Covered Person generally has no obligation to file a claim, but is responsible for presenting a current identification card and paying any applicable copay, Deductible, or coinsurance at the time of service.

Out-of-Network Providers. Out-of-network providers may or may not file claims on behalf of the Covered Person. Where the Covered Person is required to pay the provider directly and file for reimbursement, the claim must be submitted to the carrier on the carrier's designated claim form (or through the carrier's online member portal) together with an itemized bill showing the provider's name and tax identification number, the date of service, the diagnosis and procedure codes, the charge for each service, and proof of payment.

Filing Deadline. Claims for benefits under the Plan must be submitted to the carrier within 12 months of the date the Covered Service was rendered. Claims received after this deadline may be denied for untimely filing, unless the Covered Person establishes to the carrier's satisfaction that it was not reasonably possible to submit the claim within the deadline and that the claim was submitted as soon as reasonably possible.

Coordination of Benefits Information. The Covered Person must furnish, at the time a claim is filed or upon subsequent request by the carrier, information sufficient to permit coordination of benefits under Section 8, including information about other health coverage, Medicare enrollment, workers' compensation claims, motor-vehicle no-fault benefits, and third-party liability.

7.2 Initial Claim Determination Timeline

The carrier will decide claims and communicate the decision to the Covered Person (and, where applicable, the provider) within the following maximum timeframes, measured from receipt of the claim:

- **Urgent Care Claims:** as soon as possible, but not later than 72 hours.
- **Pre-Service Claims (non-urgent):** within 15 days, with a possible 15-day extension for matters beyond the carrier's control.
- **Post-Service Claims:** within 30 days, with a possible 15-day extension.
- **Concurrent Care Decisions:** decisions to reduce or terminate an ongoing course of treatment previously approved will be communicated in advance sufficient to permit appeal before the reduction or termination takes effect; requests to extend an ongoing course of urgent care will be decided within 24 hours if the request is made at least 24 hours before the end of the approved course.

Extensions require written notice to the Covered Person prior to the expiration of the initial period, stating the reason for the extension and the date by which a decision is expected.

7.3 Adverse Benefit Determination and Notice

If a claim is denied in whole or in part, the carrier will provide the Covered Person with a written notice of adverse benefit determination that includes:

- the specific reason or reasons for the determination;
- reference to the specific Plan provisions on which the determination is based;
- a description of any additional material or information necessary to perfect the claim, and an explanation of why such material or information is necessary;
- a description of the Plan's appeal procedures and applicable time limits;

- a statement of the Covered Person's right to bring civil action under ERISA section 502(a) following an adverse determination on appeal;
- if the determination was based on an internal rule, guideline, protocol, or similar criterion, either a copy of the rule or a statement that a copy is available free of charge on request;
- if the determination was based on medical necessity or experimental treatment, either an explanation of the scientific or clinical judgment applying the terms of the Plan to the Covered Person's medical circumstances, or a statement that such explanation is available free of charge on request; and
- the availability of, and contact information for, any applicable office of health-insurance consumer assistance or ombudsman.

7.4 Internal Appeal

A Covered Person (or an authorized representative) who receives an adverse benefit determination may appeal the determination to the carrier by submitting a written request for review within 180 days of the date of the notice. The request should include the Covered Person's identification, the claim in question, and any additional information, records, or arguments the Covered Person wishes to have considered.

The carrier will decide the appeal within the following maximum timeframes:

- **Urgent Care Appeals:** as soon as possible, but not later than 72 hours from receipt of the appeal.
- **Pre-Service Appeals:** within 30 days.
- **Post-Service Appeals:** within 60 days.

The appeal will be reviewed by a person or persons not involved in the original determination and not subordinate to the original decision-maker. Where the appeal turns on a medical judgment, the reviewer will consult with a healthcare professional who has appropriate training and experience in the relevant field and who was not consulted in the original determination.

The Covered Person is entitled, upon request and free of charge, to reasonable access to and copies of all documents, records, and other information relevant to the claim, including any new or additional evidence considered, relied upon, or generated by the carrier in connection with the appeal.

7.5 External Review

Following exhaustion of the internal appeal process, or where required by law without exhaustion (including for urgent care matters), a Covered Person is entitled to request external review by an independent review organization ("IRO") in accordance with the external review procedures established under the Affordable Care Act and applicable Texas insurance law. A request for external review must generally be filed within four months of the date of the final adverse determination on internal appeal. The IRO's decision is binding on the Plan and the carrier, subject to the Covered Person's right to pursue other remedies available under state or federal law.

7.6 Exhaustion and Civil Action

Except where external review is available without prior exhaustion, a Covered Person must exhaust the internal appeal process described in Section 7.4 before bringing a civil action under ERISA section 502(a). The statute of limitations for any action to recover benefits under the Plan is the shorter of the period provided by applicable law or three years from the date of the final adverse determination on internal appeal.

7.7 Authorized Representatives

A Covered Person may designate an authorized representative to act on his or her behalf in connection with a claim or appeal by submitting a completed authorization form to the carrier. Except for urgent care claims (where a treating healthcare professional is automatically deemed authorized to act on the Covered Person's behalf), the carrier may require the Covered Person's written consent before communicating with a representative.

Section 8. Coordination of Benefits

Where a Covered Person is also covered under one or more other group health plans (including another employer's group health plan, a spouse's plan, Medicare, TRICARE, Medicaid, or motor-vehicle no-fault coverage), benefits under this Plan will be coordinated with benefits under such other coverage so that the aggregate benefits from all coverages do not exceed 100% of the allowable expenses for Covered Services.

Order of Benefit Determination. The following order applies in determining which plan pays first (the "Primary Plan") and which pays second (the "Secondary Plan"):

1. A plan that covers the person as an employee (or as a subscriber) pays before a plan that covers the person as a dependent.
2. For a dependent child covered under both parents' plans, the plan of the parent whose birthday falls earlier in the calendar year pays first (the "birthday rule"). If the parents have the same birthday, the plan that has covered the parent longer pays first.
3. For a dependent child of divorced or separated parents, the order of benefit determination follows any applicable court order; in the absence of a court order, the plan of the parent with custody pays first.
4. A plan that covers a person as an active employee (or dependent of an active employee) pays before a plan that covers the person as a laid-off or retired employee, unless federal law provides otherwise (as with Medicare and TRICARE).
5. Where none of the foregoing rules determines the order, the plan that has covered the person longer pays first.

Subrogation and Reimbursement. Where a Covered Person suffers an injury or illness for which a third party is or may be liable, the Plan is subrogated to the Covered Person's rights of recovery against the third party to the extent of benefits paid by the Plan, and the Covered Person shall reimburse the Plan for benefits paid out of any recovery obtained from the third party (whether by settlement, judgment, or otherwise), regardless of whether the Covered Person is "made whole" and without reduction for the Covered Person's attorney fees, unless the Plan Administrator agrees otherwise in writing. The Covered Person shall cooperate with the Plan and the carrier in the pursuit of subrogation and reimbursement.

Medicare. Where a Covered Person is entitled to Medicare, coordination follows the Medicare Secondary Payer rules. For an active employee of a group health plan of an employer with 20 or more employees (or for the active employee's spouse), the group plan is Primary and Medicare is Secondary. Because Troutly Analytics employs 20 or more employees, this Plan pays primary for active employees and their spouses entitled to Medicare, and Medicare pays secondary in accordance with the Medicare Secondary Payer regulations.

Overpayments. The Plan is entitled to recover any overpayment resulting from coordination of benefits, including by offset against future benefits, by direct billing of the Covered Person, or by any other means permitted by law.

Section 9. Definitions

The following terms, when capitalized in this document, have the meanings set forth below. Terms defined in the Certificate of Coverage and not defined here have the meanings given in the Certificate of Coverage.

Affordable Care Act. The Patient Protection and Affordable Care Act of 2010, as amended, and the regulations issued thereunder.

Allowed Amount. The maximum amount on which payment is based for a Covered Service. For in-network services, the Allowed Amount is the amount agreed to by the network provider in the provider's participation contract with the carrier. For out-of-network services, the Allowed Amount is determined under the Certificate of Coverage.

Balance Billing. The practice by an out-of-network provider of billing the Covered Person for the difference between the provider's charge and the Allowed Amount, except where prohibited by the No Surprises Act or other applicable law.

Certificate of Coverage. The certificate, booklet, or evidence of coverage issued by the insurance carrier under the group insurance contract, together with any riders, endorsements, or amendments issued by the carrier and any summary of benefits and coverage required by the Affordable Care Act.

COBRA. Title X of the Consolidated Omnibus Budget Reconciliation Act of 1985, as amended. Troutly Analytics is subject to federal COBRA; references in this document to "COBRA-equivalent" mean continuation coverage under COBRA.

Coinsurance. The percentage of the Allowed Amount for a Covered Service that a Covered Person is required to pay after any applicable Deductible has been satisfied.

Copay. The fixed dollar amount that a Covered Person is required to pay at the time a Covered Service is rendered.

Covered Person. An employee or eligible dependent who is enrolled under the Plan and whose coverage is in effect.

Covered Service. A service, supply, or item that is:

- eligible for coverage under the Certificate of Coverage;
- medically necessary as determined by the carrier;
- rendered while the Covered Person is enrolled and coverage is in effect;
- not subject to any Plan exclusion or limitation; and
- billed and coded in accordance with the carrier's standards.

Deductible. The amount a Covered Person must pay for Covered Services in a Plan Year before the Plan begins to pay Coinsurance.

Dependent. An eligible spouse or child, as defined in Section 2.4.

Effective Date of Coverage. The date on which a Covered Person's coverage under the Plan begins, determined in accordance with Section 2.3, Section 2.5, or Section 3.4, as applicable.

Eligible Employee. An employee of the Plan Sponsor who is in an eligible class under Section 2.1 and who satisfies the hours threshold in Section 2.2.

Employer. Troutly Analytics, Inc.

ERISA. The Employee Retirement Income Security Act of 1974, as amended.

Family. For purposes of the coverage tiers in Section 3.1, the employee, the employee's lawful spouse, and one or more eligible children.

HIPAA. The Health Insurance Portability and Accountability Act of 1996, as amended.

Medical Necessity / Medically Necessary. A service, supply, or item is medically necessary if it is:

- appropriate and consistent with the diagnosis of the treating provider and could not be omitted without adversely affecting the Covered Person's condition or the quality of medical care;
- provided in accordance with generally accepted standards of medical practice in the United States;
- not primarily for the convenience of the Covered Person, the provider, or any other person; and
- the most appropriate level of service that can be safely provided to the Covered Person.

Determinations of medical necessity are made by the carrier in accordance with its clinical criteria, subject to the appeal rights in Section 7.

Network / In-Network Provider. A physician, hospital, pharmacy, or other provider that has entered into a written contract with the carrier or its designated network to accept the Allowed Amount as payment in full for Covered Services rendered to Covered Persons, less any applicable Deductible, Copay, or Coinsurance.

No Surprises Act. Title I of Division BB of the Consolidated Appropriations Act, 2021, and the regulations issued thereunder, which limit balance billing for certain out-of-network emergency services, non-emergency services at in-network facilities, and air-ambulance services.

Open Enrollment. The annual period during which eligible employees may enroll, disenroll, or change coverage, as described in Section 3.3.

Out-of-Network Provider. A provider that is not an In-Network Provider.

Out-of-Pocket Maximum (OOPM). The maximum amount a Covered Person or family will pay in Deductibles, Copays, and Coinsurance for in-network Covered Services in a Plan Year, as described in Section 5.3.

Plan. The Troutly Analytics, Inc. Group Medical Plan, as described in this document and funded through the group insurance contract.

Plan Administrator. The Head of People of Troutly Analytics, Inc., as described in Section 1.5.

Plan Sponsor. Troutly Analytics, Inc.

Plan Year. The 12-month period beginning 09/01 and ending 08/31, as described in Section 1.4. The Plan Year covered by this document runs from 09/01/2025 to 08/31/2026.

Prior Authorization. The requirement that a Covered Person or provider obtain the carrier's approval before a specified service is rendered, in accordance with the carrier's clinical policies.

Qualifying Life Event. An event described in Section 3.4 that permits a Covered Person or eligible employee to enroll or change coverage outside of Open Enrollment.

QMCSO. A Qualified Medical Child Support Order within the meaning of ERISA section 609.

Rescission. A cancellation or discontinuance of coverage that has retroactive effect, other than a cancellation or discontinuance that has only a prospective effect or that is attributable to a failure to timely pay premium.

Spouse. The employee's lawful spouse under the law of the state where the marriage was contracted, including a common-law spouse recognized under applicable state law upon submission of appropriate documentation.

Summary Plan Description (SPD). The description of the Plan required to be furnished to participants under ERISA section 102, of which this document forms a part.

United States Preventive Services Task Force (USPSTF). The independent panel of national experts in prevention and evidence-based medicine whose recommendations at Grade A or B are required to be covered without cost-sharing under the Affordable Care Act.

Waiting Period. The period of 30 days of eligible employment that must be completed before coverage becomes effective, as described in Section 2.3.

Section 10. Plan Administration and Required Notices

10.1 Plan Administration

The Plan Administrator, as identified in Section 1.5, is responsible for the general administration of the Plan, including eligibility determinations, enrollment processing, contribution collection, communications with the insurance carrier and the broker of record, and the discharge of the Plan Sponsor's reporting and disclosure obligations under ERISA. The Plan Administrator may delegate administrative functions to service providers, including the insurance carrier and Lampasas Benefits Group, and may adopt written procedures for the administration of the Plan.

10.2 Amendment and Termination

The Plan Sponsor reserves the right, in its sole discretion, to amend or terminate the Plan in whole or in part at any time by action of its Board of Directors or a duly authorized officer. No amendment shall reduce benefits with respect to Covered Services rendered prior to the effective date of the amendment. On termination of the Plan, coverage will end as of the date of termination, subject to any continuation rights required by law and to the terms of the insurance contract.

10.3 Statement of ERISA Rights

As a participant in the Plan, a Covered Person is entitled to certain rights and protections under ERISA, including:

- the right to examine, without charge, at the Plan Administrator's office, all documents governing the Plan and a copy of the latest annual report (Form 5500 Series) filed with the Department of Labor;
- the right to obtain, upon written request to the Plan Administrator, copies of documents governing the operation of the Plan and copies of the latest annual report (a reasonable charge may be made for copies);
- the right to continue health-care coverage as required by law upon the occurrence of a qualifying event, as described in Section 6.1; and
- the right to file suit in a federal or state court if a claim for benefits is denied or ignored, in whole or in part, after exhausting the Plan's appeal procedures.

Fiduciaries of the Plan have a duty to operate the Plan prudently and in the interest of the Plan's participants and beneficiaries. No one, including the Plan Sponsor, may fire a participant or otherwise discriminate against a participant in any way to prevent the participant from obtaining a welfare benefit or exercising rights under ERISA.

For assistance in understanding these rights, a Covered Person may contact the Plan Administrator, the nearest office of the Employee Benefits Security Administration of the United States Department of Labor, or the national office of the Employee Benefits Security Administration.

10.4 HIPAA Privacy

The Plan maintains a Notice of Privacy Practices describing the uses and disclosures of protected health information permitted under the Health Insurance Portability and Accountability Act and the regulations issued thereunder. The Notice of Privacy Practices is available from the Plan Administrator on request and is furnished to Covered Persons in accordance with applicable law.

10.5 Women's Health and Cancer Rights Act

The Plan covers, for any Covered Person receiving benefits in connection with a mastectomy and who elects breast reconstruction, all stages of reconstruction of the breast on which the mastectomy was performed, surgery and reconstruction of the other breast to produce a symmetrical appearance, prostheses, and treatment of physical complications of the mastectomy, including lymphedema. These benefits are subject to the same Deductible and Coinsurance applicable to other Covered Services under the Plan.

10.6 Newborns' and Mothers' Health Protection Act

The Plan does not restrict benefits for any hospital length of stay in connection with childbirth for the mother or newborn child to less than 48 hours following a vaginal delivery, or less than 96 hours following a cesarean section, or require that a provider obtain authorization from the carrier for prescribing a length of stay not in excess of the above periods.

10.7 Michelle's Law and Children's Health Insurance Program Reauthorization Act

The Plan complies with Michelle's Law (with respect to continued coverage of dependent children on medically necessary leave from post-secondary education) and CHIPRA (with respect to special enrollment on loss of Medicaid or CHIP coverage or on gaining eligibility for state premium assistance).

10.8 No Guarantee of Employment

Participation in the Plan does not give any employee or dependent a right to continued employment with the Plan Sponsor, and the terms of the Plan may not be construed to limit the Plan Sponsor's right to terminate the employment of any employee at any time, with or without cause, subject to applicable law.

10.9 Governing Documents

In the event of any conflict between this document and the Certificate of Coverage on a matter of benefit determination, the Certificate of Coverage controls. In the event of any conflict between this document and any communication distributed to employees on a matter of eligibility, enrollment, or contribution, this document controls. Oral representations by any employee of the Plan Sponsor, the carrier, or the broker of record are not binding.

Prepared for Troutly Analytics, Inc. by Lampasas Benefits Group for the Plan Year 09/01/2025 to 08/31/2026.